

Mental Health Quackery

In 2003, New York matrimonial attorney Timothy Tippins published in the *New York Law Journal* a cautionary article on the use of mental health theories in family courts. Tippins, former chair of the New York State Bar Association's Family Law Section and Task Force on Family Law, first stated the obvious: "the role of mental health opinion in court has proliferated in recent decades and nowhere is its impact more profound than in child custody litigation." Then he gave this warning: Mental health opinions can spell the end of the line for the disfavored parent because of the indicia of authoritativeness they have come to carry . . . that emanates from the assumption that the opinion rests on scientifically reliable and valid principles and procedures. Sadly, that assumption is unwarranted. . . [S]ome courts are deeply influenced by these opinions and reach custodial decisions unaware that scientifically they stand on feet of clay.¹ If the reliance of courts on meretricious "science" poses a problem for "the disfavored parent" in ordinary custody litigation, it is downright dangerous in cases involving child sex abuse allegations. In these cases, pseudoscience regularly does serious damage to mothers who raise such allegations and to the children who may be victims. In fact, hostile epithets posing as scientific "syndromes" have played a central role in the backlash of the family courts against protective parents. Branded as "vindictive," "calculating," "manipulative" and/or "hysterical," "narcissistic," or "paranoid" on the basis of one questionable theory or other, women lose custody of their children, and, in many cases, all visitation rights as well.

The importance of these pseudoscientific labels in a closed system like the family courts can hardly be exaggerated. They establish a “running record”—an informal history that is invoked at critical points in the decision-making process, eclipsing every other feature of the case, including evidence that the mother in question is functional and rational. They may even overshadow objective evidence of possible sexual abuse to a child. Mothers dogged by these court-generated “running records” find that nothing they say is taken seriously—except as further “evidence” against themselves. Yet their only “sickness” may be their attempt to protect their children from abuse.

Specious “mental illness” labels applied to protective mothers can corrupt the judicial process in several ways. For one thing, they can cause family courts to overlook evidence of abuse because that evidence dovetails with “symptoms” of one of the mental health theories whose effect is to discredit accusing mothers. Second, they allow courts to punish mothers for making sex abuse allegations that are determined to be *unproven* as if the allegations were found to be *maliciously fabricated*, by labeling the accusation as part of a mental illness that will supposedly cause the mother to continue a “pattern of denigration” if she is not deprived of custody and otherwise penalized. Third, they can feed a system that is already suspicious of protective mothers’ “psychological” reasons to believe that the mother will kidnap her children and flee the jurisdiction, reasons such as claiming her personality is “grandiose” or “paranoid.” Fourth, such theories can cause courts to do precisely what is most dangerous to abused children; that is, they can cut the children completely off from the parent who believes them and wants to protect them from the abuse they report.

Most of all, perhaps, mental health quackery functions as the theoretical glue that holds together all the elements of malfunction we have been examining: judicial overreaching, unregulated law guardians and social service agencies, misplaced priorities, systemic prejudices, and unchecked power.

Genuine experts have already noted some of the dangers inherent in introducing psychological theories into domestic litigation. Psychiatrist Jonathan W. Gould, in *Conducting Scientifically Crafted Child Custody Evaluations* (1998), specifically cautioned judges against overreliance on an expert's opinions, giving undue weight to the personal opinions of a witness offered within the guise of "scientifically based opinion."² But despite the caveats expressed by Gould and others, mental health professionals enjoy an expanding role in family court litigation. Not only are their opinions given great weight in evaluations of abuse charges—in some cases, their role extends even beyond the evaluative stage. For example, they can act as court-appointed visitation supervisors (supervising a mother's visits with her children) and as court-appointed family counselors, even after the litigation is over). They can also serve as consultants to Child Protective Services (CPS), which means that they can recommend that charges be filed against a mother they deem "paranoid" or "delusional." These various roles multiply the effect of the theories that such "experts" carry into each particular case. We can see, then, that family court madness depends critically on the theories of psychologists who function within the system—theories that, significantly, have little or no life outside the family courts, seldom passing muster under the accepted standards of mental health professions. And by applying ethnomethodology, we further learn how it is that mental health experts in their day-to-day work—giving undeserved diagnoses to protective mothers—reflexively *re-create* a mad system that in turn deems their practice an acceptable mode of operation. In this chapter we examine the foundations of quack theories masquerading as mental health theories in the family courts and the effects they have on judicial decision making.

THE THEORIES

In Chapter 2, we touched on the most prominent of the questionable mental health theories used in family courts. A brief recapitulation will be useful here.

New Jersey psychiatrist Richard A. Gardner's "Parental Alienation Syndrome" (PAS) is perhaps the best known, and most widely used, of these family court theories. Gardner claimed that he had identified a "disorder" in which "children, programmed by the allegedly 'loved' parent, embark on a campaign of denigration of the allegedly 'hated' parent." Extensive criticism of Gardner's work by mental health professionals, legal scholars, and some judges has done little to diminish the popularity of PAS in the family courts.

In a 1998 law review article thoroughly analyzing Gardner's theory, Kathleen Niggemyer pointedly observed that although PAS

has not been subjected to peer review or accepted by experts in the fields of psychology or child advocacy . . . some courts have used Gardner's PAS theory to quickly diagnose PAS, abruptly remove a child from a custodial parent who alleges abuse, and place the child in the custody of the allegedly abusing parent simply because the custodial parent has alleged that the non-custodial parent was abusing the child.³

In 1996, the American Psychological Association's Presidential Task Force on Violence and the Family, in a report titled "Violence and the Family," had specifically rejected Gardner's theory. Criminal courts have done so as well. For example, in 1999, a New York court refused to admit evidence of PAS in defense of a man accused of criminal child abuse, finding that PAS was not "a generally accepted concept with the professional community."⁴ More recently, a California appeals court upheld a criminal court's exclusion of testimony about PAS on the ground that

“the testimony was not scientific enough to satisfy the ‘*Kelly-Frye*’ rule,” which is the “gold standard” for the admissibility of evidence.⁵

But the reception given to Gardner’s theory by family courts has been astonishingly different. Indeed, although Richard Gardner died in 2003, PAS continues to have an amazing influence in the family court system both in the United States and abroad.⁶ For example, several months after Gardner’s death, a mother lost custody of her two sons, aged nine and eleven, in an Israeli court that based its decision on Gardner’s theories. An Israeli newspaper that covered the trial quoted Dr. Paul Fink, a professor of psychiatry at Temple University and past president of the American Psychiatric Association, on the impact of Gardner’s theories in this case: “This is junk science. . . Dr. Gardner should be a rather pathetic footnote or an example of poor scientific standards.”⁷ Obviously, this was not the opinion of the court that made its custody decision on the strength of Gardner’s theory.

Until his death in 1998, Dr. Arthur Green was, like Gardner, a highly popular evaluator in cases of alleged child abuse. Like Gardner, Green thought protective mothers should be carefully examined for evidence of “brainwashing.” For instance, Green advocated closely watching the mother-child interaction when a child reports abuse. He opined that if the child looks at his mother while describing being sexually abused to a professional, this indicates a “brainwashed” child. He also argued—as we shall see, with fateful consequences—that a protective mother may be paranoid and delusional with respect to an allegation of sex abuse while remaining completely functional, without any trace of mental illness, in all other areas.

Both these (typical) claims of Dr. Green have been strenuously criticized by his colleagues outside the family court system. One outspoken New York judge, Jacqueline Silbermann, harshly characterized both Green and Gardner in 1991 after the two of them teamed up⁸ to condemn a protective mother in a case litigated before her. In deciding the case, Judge Silbermann stated:

Dr. Green and Dr. Gardner's testimony was that of the worst hired guns, each showing a willingness to opine on virtually anything petitioner [the father] requested them to. Each was willing to criticize every other professional in the case, and yet neither of them verified any information or any history given to them by petitioner.⁹

But Silbermann's repudiation stands alone; Green remained a popular and prestigious expert witness until he died in 1998. What is more, his methods are still used by other family court evaluators. Psychologist Ira Turkat, writing along the lines laid down by Gardner and Green, has named "Malicious Mother Syndrome" (MMS) to identify "mothers [who] not only try to alienate their children from their fathers, but are committed to a broadly based campaign to hurt the father directly."¹⁰ The use of this theory in family courts has been severely criticized by women's advocates. For instance, Smith and Coukos (1997) stated: "Courts should be vigilant in evaluating any psychological expert testimony that claims to be able to discern false from true allegations, or that can be used to explain away or cover up abuse," concluding that the "most well-intentioned judges may be completely unaware of how they [judges] view protective parents until they are presented with empirical information about . . . child abuse."¹¹ This advice, however, has not been heeded by many family court judges.

Below we dissect the significant elements of these suspect theories and discuss as well how they actually function in family courts.

WHY DO FAMILY COURTS USE QUACK THEORIES?

Family courts are charged with an extremely difficult and often unrewarding task in cases involving child abuse allegations. Such charges are very difficult to prove, and available evidence is often available only in the form of opinion. Complicating the matter is the fact that the opin-

ions of experts may easily conflict in the same case. One expert may find a child's disclosure of abuse persuasive—another may have doubts about it. To make matters even more difficult, repeated interviews of the child—which are nearly always necessitated by the court process—make evaluation more difficult as the child's utterances become less spontaneous. And yet no judge can conscientiously ignore charges of child abuse. So, the family court judge in such cases is repeatedly called on to make a Solomonic choice, poised between the knowledge that a rush to judgment may devastate an innocent parent and the fear that failure to find abuse when it is actually occurring will betray everyone involved, especially the child. And the judge must often make such a choice with very little real help.

The difficulty of evaluating children's disclosures accurately is well recognized in the relevant literature. In 1987, in the *Journal of Interpersonal Violence*, psychiatrist David Corwin, along with other researchers, pointed out "the well-known tendency of abused children to utilize denial and dissociation in defending against their memories of abuse [which] could create a range of unpredictable and seemingly paradoxical reactions."¹²

Roland Summit, professor of psychiatry at the University of California (Los Angeles), has introduced the term "Child Abuse Accommodation Syndrome" to describe a number of behavioral responses to sexual abuse trauma—low self-esteem, helplessness, depression, and self-blame. Dr. Summit emphasizes that, as part of this syndrome, children may "recant" their allegations of abuse and even show signs of affection toward their abuser.¹³

Researchers S. E. Palmer and R. A. Brown, in a 1999 study of 116 cases of child sexual abuse that had been confirmed by a perpetrator's subsequent guilty plea or conviction (or by highly consistent medical evidence), found that 72 percent of the child victims actually denied the abuse when they were first questioned about it. The Arizona Coalition against Domestic Violence, in its 2003 study of protective mothers, cites

the Palmer and Brown study to show that “victims don’t disclose [abuse] because of fear of the consequences, self-blame, lack of awareness and difficulty in talking about the abuse.”¹⁴

Yet honest family court judges are expected to sift these paradoxical, inconsistent facts and statements and make decisions that will radically alter a family’s life. It must be admitted that the judge’s task in such cases is not enviable.¹⁵

Compared with such daunting responsibility, it is fatally easy to blame the accuser. This not only disposes of the case—obviously there is no child abuse if a “delusional” parent says there is—it eases the conscience of a judge who might otherwise worry about making the wrong decision. If a mother is crazy and “vindictive,” then the court’s obligation is clear. That the course taken by the court in such cases—silencing the accuser, cutting off contact between the child and anyone who believes abuse is occurring—is precisely what will protect a father who really is guilty of abuse rarely seems to trouble judicial minds.

Thus, family courts have come to rely on psychological theories that, however poorly substantiated scientifically, do a fine job of diverting the court’s attention from the alleged abuse to the personality of the mother. New Jersey psychiatrist Richard A. Gardner, one of the most notorious critics of protective mothers in child abuse/custody litigation until his suicide in 2003, provides an excellent example of this. In his writing, Gardner set forth some extraordinary characterizations of mothers who made what he termed “false” reports of abuse.¹⁶ For example, in *True and False Accusations of Child Sex Abuse* (1992), he stated that “mothers who fabricate a sex abuse allegation are often hysterical and/or exhibitionistic. They typically exaggerate situations, ‘make mountains out of molehills,’ and will take every opportunity to broadcast the abuse.”¹⁷ It can readily be seen that when such a theory is applied to a mother—any mother—who believes her child has been abused, her anger (“hysteria”), anxiety about possible repetition of the abuse based on whatever evidence she may have (“exaggerating situations”), and repeated calls for

help (“broadcasting the abuse”) can quickly type her as a “falsely accusing” mother.

Another controversial evaluator, Dr. Arthur Green, fits the same blame-the-accuser model. In 1986, for instance, Green advocated closely watching the mother-child interaction when a child reports abuse to a professional, claiming “delusional and vindictive mothers often control the child by monitoring his or her responses through eye contact and subtle facial expressions. The ‘brainwashed’ children respond by ‘checking’ with their mothers before proceeding.”¹⁸ The application of this theory, too, can easily stamp any disclosure of child abuse a “false” one. Indeed, many of Green’s colleagues sharply disagreed with the approach set out in this article. Two years after it appeared, Graeme Hanson, assistant clinical professor in psychiatry and pediatrics at the University of California, San Francisco, and eighteen cosigners published a letter in *Journal of the American Academy of Child and Adolescent Psychiatry* in which they stated: “In our experience, children who are anxious . . . ‘check’ with their mothers for reassurance before proceeding. This does not invariably indicate brainwashing.”¹⁹

Green’s insistence that a mother’s “subtle” encouragement of her child’s sex abuse report marks her as “delusional and vindictive” is particularly ominous because many children are reluctant to disclose abuse, even to professionals, without a certain amount of encouragement. In a 2001 study of sixty-four non-offending mothers of children believed by researchers to have been abused, Deblinger and coauthors reported that most abused children will not talk about sexual abuse if they are not specifically prompted to do so (as in, for instance, a discussion about “good touch” and “bad touch.”)²⁰

This means that a protective mother, confronted by an evaluator like Dr. Green, faces a Hobson’s choice: since abused children are unlikely to initiate a conversation about abuse, a worried parent must in some sense “prompt” the child by introducing the topic of abuse. But this will

open the mother to a charge that she “coached” her child into fabricating sex abuse. And if a Dr. Green has his way, she is likely to lose custody.

Indeed, both Gardner and Green sometimes went to astonishing lengths to discredit protective mothers’ allegations of sexual abuse. For example, they argued that sex abuse accusations endorsed by a mother during a contentious divorce actually express her own sexual problems—including her alleged sense of sexual deprivation.²¹ Gardner never produced any evidence to support this peculiar theory, but he was perfectly clear about what he meant:

[A] mother . . . who cannot allow herself to accept the fact that she still may harbor the desire for sexual activities with her now-hated spouse, may project these feelings onto her daughter and develop the delusion that her husband has sexually molested the child. It is as if each time she entertains the pedophilic fantasy (of her husband molesting her daughter), she satisfies vicariously (by projecting herself into her daughter’s position) her own desire to be the recipient of her spouse’s sexual overtures.²²

Green made a similar claim. In describing the characteristic situations that breed false disclosures of sexual abuse by a child, he stated:

[T]he child is influenced by a delusional mother who projects her own unconscious sexual fantasies onto the spouse . . . the allegations of incest usually begin shortly after the marital separation, while the child is visiting the father. The object loss inherent in the separation and impending divorce acts as the catalyst for this pathological process.²³

Here, clearly, is an argument that attributes any abuse accusation surfacing during divorce to unresolved sexual problems of the mother—not the accused father!—and thus turns the normal sex abuse inquiry

on its head. Indeed, such attacks on protective mothers, posing as sober scientific insights, illustrate a particularly pernicious aspect of family court quackery: precisely those things that tend to appear in meritorious sex abuse allegations can be used, in light of these theories, to discredit them.

As a matter of fact, a large body of peer-reviewed literature shows why sexual abuse incidents are likely to occur at the time of the divorce—a phenomenon that cannot be explained away as the “projection” of fantasies by a sexually frustrated wife.

For example, as David Corwin and his coauthors have argued, “the losses, stresses, and overall negative impact of separation and divorce may precipitate regressive ‘acting out’ by parents, including child sexual abuse.” The authors further explore the correlation between divorce and sex abuse by suggesting that

it is possible that the adult character traits and behavior problems frequently associated with the sexual abuse of children are more common in people whose marriages break up. Included in this list are narcissistic traits . . . paranoid ideation . . . antisocial tendencies, impulsivity . . . sexual difficulties . . . and substance abuse.²⁴

Maryland psychologist Dr. Mary Froning, in a September 1988 letter to the *Journal of the American Academy of Child and Adolescent Psychiatry*, cited Professor Kathleen Faller’s published work to stress

three factors that seem to contribute to the risk of sexual abuse in divorces, especially when the abusing parent resisted the divorce. First, is the loss of structure of family life; second, the feelings of devastation and loneliness on the part of the offender, which cause him to turn to the child for emotional support and then lead to sexual interaction with the child; and

third, the tendency of the perpetrator to express some of his anger toward the spouse indirectly by sexually abusing the child.²⁵

Corwin and coauthors also noted that “abused children may be more likely to disclose abuse by a parent and to be believed by the other parent following separation and divorce.” They explain: “[W]ith the breakup of the parents comes diminished opportunity for an abusing parent to enforce secrecy as there is increased opportunity for the child to disclose abuse separately to the other parent.”²⁶ In other words, sex abuse is more likely to be disclosed by the child who feels safe enough to talk about the abuse when the abuser is already out of the house. In light of all this published work pointing to the greater likelihood of abuse disclosures during divorce, it is clear why Green’s and Gardner’s theorizing—which goes so far as to blame such disclosures on an unstable, vindictive, and sex-starved ex-wife—is likely to turn judicial inquiry in an abuse case onto a course diametrically opposed to its proper object. Sadly, that is precisely what makes such theorizing popular in family court.

ALL MOTHERS SUFFER FROM THE SAME ILLNESS

The psychological theories unique to family courts have several unusual features. These features deserve analysis because a detailed examination helps to explain the damage these unsubstantiated theories can do in cases involving child abuse allegations.

The first odd feature of the theories we are examining is that they place *all mothers* the theorists wish to stigmatize as “false reporters” in the same procrustean bed. In the population at large there are many kinds of mental illness, each consisting of basic pathological conditions with a wide range of accompanying features. In the family courts, however, mothers across the country are diagnosed as suffering from the

same genre of mental illnesses. This, of course, defies statistical probabilities and common sense.

Protective mothers are described by these “experts” in highly predictable language. In lay terms they are “self-absorbed,” “manipulative,” “calculating,” “rigid and judgmental,” and “overly opinionated.” In psychiatric terms they are “*paranoid-delusional*,” with “*narcissistic*,” “*histrionic*,” and “*exhibitionist*” features, and display “*grandiose*” self-assessments. Many mothers trying to prove their charges of sexual abuse are found by mental health experts to be living in a “fantasy” world where they feel compelled to superimpose *their* beliefs on everyone around them.

These terms reappear with depressing frequency in “expert” evaluations rendered in family courts. It should be borne in mind that a mother who believes her child has been abused is likely to be emotional; that if the charge is not readily acted on, she may well tend to suspect (often, alas, with good reason) that officials connected with the case are favoring the accused abuser; that in order to protect her child she is obliged to try to make other people accept her belief that a danger exists. Thus, a mother with a well-founded suspicion of abuse may easily be pigeonholed as “mentally ill” if family courts accept these boilerplate evaluations, as they all too often do.

“MENTAL ILLNESS” THAT IS “CIRCUMSCRIBED” TO ONE TOPIC

A particularly strange feature of family court mental illness theories is that they provide for mothers to be “paranoid” and “delusional” about one issue—their suspicion of sexual abuse—while showing no trace of mental illness in any other connection. Mothers have been found “paranoid” by court-appointed experts solely because of their belief in their children’s abuse. Yet in many of these cases, the “paranoid” mother is embarrassingly devoid of some of the most common symptoms of para-

noia, such as intense and irrational mistrust or suspicion (which can bring on a sense of rage, hatred, and betrayal), a preoccupation with hidden motives, a fear of being deceived or taken advantage of, and the inability to relax. True paranoids are argumentative, abrupt, stubborn, self-righteous, and perfectionistic. Protective mothers often display little or nothing of this clinical profile.

Arthur Green, the New York psychiatrist whose theories drew sharp criticism from his colleagues (as we previously discussed), dodged this fatal objection by claiming in a 1986 article that such mothers suffered from a unique sort of “paranoid psychosis” that was “circumscribed” or restricted to a single topic. He illustrated this so-called diagnosis with an account of one case in which he had served as court-appointed evaluator.²⁷

About a year later, Green claimed to locate just such a “circumscribed paranoid psychosis” in another case. The mother in question, whose “paranoia” consisted in the fact that she believed her mother’s eyewitness account that her six-year-old daughter had been sexually abused—an abuse incident that one of New York’s most prominent abuse evaluators found “strong reason to believe” after interviewing the girl—had already been examined by a psychiatrist and found to have no mental illness. Green himself, in a report submitted to the family court, admitted that the mother was “fully adjusted” and “performing successfully” in her professional and personal life. He even admitted that she did not suffer from any delusions—except for her beliefs concerning her daughter.²⁸ Nevertheless, he did not hesitate to label her belief in her daughter’s abuse a “circumscribed psychosis.” In his 1986 article, Green had given the same diagnosis to “Betty C,” the mother whose case he had used to illustrate a “false” allegation of sexual abuse. She, too, was a competent professional woman who demonstrated a high level of functioning.²⁹

The legal danger of pronouncing a single belief a sign of paranoia, without considering the possibility that this particular belief may be jus-

tified, should be obvious. Green's offense in this case was particularly egregious because (as will be discussed in more detail) he concluded no abuse had occurred not from an examination of evidence but simply because the judge told him privately that it had not (while the judge, in turn, claimed to base his conclusions on Dr. Green's evaluation!).

But even Green's clinical assumption—that a “circumscribed psychosis” is a legitimate sort of diagnosis in a wide range of family court cases—is untenable. In truth, a “circumscribed” psychosis—a psychosis narrowly restricted to one topic—is an extremely rare occurrence. Most clinicians consider it nearly impossible for the same individual to have “delusional” or “paranoid” thinking about one particular topic while showing no distorted thinking about other topics. A psychiatrist consulted by an investigator for a New York State legislative committee interested in family court malfunction explained to one of us that the kinds of psychotic illnesses that cause delusional and paranoid thinking cannot “freely discriminate” among topics, “choosing one thing to be paranoid about and not another.”³⁰

Yet Dr. Green has attributed this rare condition to many mothers in family court litigation across the country. Four of these mothers later gave compelling firsthand testimony at both state and federal legislative hearings on the devastating effects of such diagnoses on their cases.³¹ But Green's many followers still find a willing audience for this theory in family courts.

MENTAL ILLNESS THAT CAN BE “INDUCED”

Another claim almost unique to family courts is that mothers who suffer from an “illness” that makes them believe their children have been abused are able to *induce* that same mental illness in their children. Mothers are accused of *inducing* paranoia in their children, who join their mothers in thinking “crazy” thoughts. Faller, Corwin, and Olafson

(1993) trace this strange idea back to 1981—and to a family court case, naturally. Two researchers (S. L. Kaplan and S. J. Kaplan)³² studied the case of two children whose mother had made allegations of sexual abuse. Believing the charges to be false (even though they were confirmed by the children themselves, who testified consistently to the abuse over many court sessions), they proposed “the possible dynamic of *folie à deux* (folly of two) as an explanation for the children’s allegations” of sexual abuse.

Faller, Corwin, and Olafson emphatically deny that such an application of the notion of *folie à deux* is clinically acceptable. They point out that Kaplan and Kaplan offered their diagnosis “despite the fact that there was no delusional thinking diagnosed in either child, the mother, or the maternal grandparents. .”³³ That is, for a child to be susceptible to “paranoid” and “delusional” thinking, both the child and the mother must have an underlying *psychosis*. No such psychosis was found in the case studied by Kaplan and Kaplan.

Despite the deeply flawed foundation of this theory, however, the *folie à deux* concept has gained wide acceptance in the family courts. In the 1980s, Dr. Richard Gardner used it as part of his “Parental Alienation Syndrome”⁴—“a disorder in which children, programmed by the allegedly ‘loved’ parent, embark on a campaign of denigration of the allegedly ‘hated’ parent,” whereby the “sex abuse accusation emerges as a final attempt to remove him [the father] entirely from the children’s lives.”³⁵

It is easy to see why Gardner embraced *folie à deux*, however rare such a phenomenon might actually be in the circumstances encountered by family courts, to buttress his “syndrome.” Gardner claimed that sex abuse allegations were being manufactured by hysterical *mothers*. However, he was faced with the embarrassing fact that in most cases in which courts consulted him the *children* also described having been abused. What was more, the children’s reports appeared sincere and credible. Gardner avoided the difficulty by introducing *folie à deux*,

arguing that the children's reports must have resulted from mental illness *induced* in them by mentally ill mothers.

In fact, Gardner's theory forced him to go even further than that. He claimed that mothers could not only induce paranoid psychotic thinking in their children (i.e., they could "program" children to believe they had been abused)—they could even induce such psychosis *in their children's therapists!* "[T]here are some [therapists]," wrote Gardner, "who are indeed man haters, and who do indeed partake in a *folie à trois* [folly of three] relationship with their patients."³⁶ In Gardner's world, a child, mother, and therapist do not necessarily all confirm an abuse report simply because it is true—this might, instead, occur as the result of a strangely contagious mental condition.

Law professor Carol Bruch, in a critique of Gardner's theories published in 2001 in the *Family Law Quarterly*,³⁷ points out two obvious flaws in Gardner's argument. First, she notes that *folie à deux* or shared psychotic disorder—a "disorder in which a second or further person in a close relationship with a primary person comes to share delusional beliefs of the primary person, who already had a Psychotic Disorder, most commonly Schizophrenia—is rare."³⁸ Second, she points out that for such disorders to "occur in primarily *young* children is also contrary to the literature."³⁹ Yet Gardner's cases mostly involved young children, and he claimed to find "induced" mental illness in case after case of alleged child abuse.

Another theory of "induced" mental illness that has flourished in family courts is called "Munchausen Syndrome by Proxy" (MSBP). This strange theory has become, according to one prominent matrimonial attorney, the "classic" countercharge to an abuse allegation in family court litigation.⁴⁰ Yet, as used in family courts, it rests on a distinctly flimsy foundation.

Munchausen Syndrome, named after the eccentric writer Baron von Münchhausen, is a mental illness characterized by a need to attract attention and sympathy by assuming symptoms of strange illnesses

or similar conditions. In 1977, a British pediatrician, Roy Meadow, first identified a variant of this syndrome that he named “Munchausen Syndrome by Proxy” (MSBP). In this syndrome, a parent—usually a mother—uses her *child* as the “proxy” by which she attracts the attention of health care professionals. Such mothers constantly subject their children to unneeded and sometimes dangerous medical treatment and will even induce symptoms in their children in order to keep doctors’ attention. The illness is considered rare but is also considered very dangerous by psychiatrists, particularly because it is difficult to diagnose: mothers who suffer from the disease generally appear to be highly conscientious caregivers.

Evidently, a theory that can stigmatize a mother who seems to be an excellent parent was too good for family court theorists to resist. The theory of “Munchausen Syndrome by Proxy” (MSBP) has gradually developed an offshoot that rarely appears outside family court litigation—but can be devastating within it. Perhaps the first step in the growth of this branch of the theory occurred when Herbert A. Schreier, a Florida psychiatrist, argued that it was specifically a women’s disease. We have noted that most diagnosed cases of MSBP do involve mothers—but Schreier insisted that there were special reasons why MSBP (also referred to as “factitious disorder by proxy”) should occur in mothers. In *International Pediatrics*, Dr. Julio Apolo cited the work of Schreier and his cowriter, J. A. Libow, for the proposition that *mothers* with a history of “emotional neglect in their early childhood especially by their fathers” turn to the physician “as the longed-for father figure and, by inducing or falsifying illness, the child is used to relate to and control the physician.”⁴¹ Of course, this hypothesis, which blandly assumes that “the physician” is always male, overlooks the large percentage of physicians today who are women—in pediatric medicine in particular as many as 50 percent in the United States are women—but this was only the beginning.

In 1986, British psychologists Sinanan and Haughton argued ominously that “the probable range of variations in the presentation of Münchausen syndrome is likely to develop in parallel with the evolution of medical and social services,”⁴² implying that the increased availability of services to sex abuse victims would result in a larger number of false claims by an adult that a child has been molested.

Sinanan and Haughton did not claim to have actually observed any such cases. But by 1993, in *Issues in Child Abuse Accusations*, California psychologist Deirdre Conway Rand was claiming to have identified a “contemporary” form of MSBP that occurs “when the mother creates the appearance that the child has been abused by someone else, generally the father in a divorce and custody or visitation dispute.”⁴³ Rand had already argued, in a 1990 article, that this peculiar affliction, like Gardner’s PAS, could be induced in a child, who would then act “as a confederate of the mother, with the two involved in a sort of *folie à deux*, a pattern that might be perpetuated even after the child reached adulthood.”⁴⁴

Apart from its reliance on the faulty *folie à deux* principle, there are important theoretical objections to this approach to MSBP. Kathleen Faller, professor of social work at the University of Michigan School of Social Work, argued in a 2002 PowerPoint presentation to the Seventh Annual Northern New England Conference on Child Maltreatment that MSBP is misapplied in protective mother cases for several reasons. For example, the classic MSBP mother “wants to be seen as super-mom . . . while moms in divorce are not seen as super-moms.” In addition, classically defined “MSBP moms do not accuse someone [of abuse].” Faller concluded that the improper use of MSBP diagnoses in custody cases “is just another way to prevent mothers from protecting their children.”⁴⁵

The wisdom of Faller’s objections can be seen by examining an application of this “contemporary” MSBP theory to an actual family court case. One example will suffice here.

A Long Island, New York, mother whose seven-year-old daughter was diagnosed at birth with spina bifida, a congenital condition that causes a child to be born with a partly exposed spinal column and results in permanent damage to certain nerves, lost custody of her child primarily because she was diagnosed with MSBP. But she was not even suspected of this “illness” until she brought sexual abuse allegations (confirmed by the child) to the attention of the family court.

For seven years, the mother was the child’s primary caretaker, working extensively with doctors and nurses in treating her daughter’s complex and chronic condition. Not one health care professional ever so much as hinted at the presence of MSBP in this mother. On the contrary, the child’s regular pediatrician—who treated her from birth until shortly before the mother lost custody—wrote to the court: [T]he mother . . . was always compliant and gave appropriate medical care. [The child] has multiple problems including hydrocephalus, seizures, spinal cord problems, and bladder problems. [The mother] always had objective findings and . . . was appropriate in bringing [the child] into the office or hospital as the situation warranted.⁴⁶

Only after the mother had accused the child’s father of sexually abusing the girl did court-appointed experts produce a diagnosis of MSBP.

A court-appointed evaluator, a PhD in psychoanalysis, reported to the family court judge that the mother was making “false” allegations of sex abuse and that this might justify a diagnosis of mental illness, including MSBP:

The false allegations of sex abuse can be seen as part of the divorce syndrome and ultimate goal of alienating the children from the father. Additionally, it could be a part of a Munchausen by Proxy Syndrome. There is reason to speculate that [the mother] has created or exaggerated medical symptoms with regard to [the child]. The current forensic evaluation does strongly speculate that the sex abuse allegations fit into a contemporary type of Munchausen by Proxy Syndrome.⁴⁷

Note that this was the first time, in the child's seven years of intensive medical treatment, that anyone had suggested that the mother had either "created" or "exaggerated" any of the child's medical symptoms. In fact, the "expert" herself claimed only that there was "reason to speculate" along these lines, and any such speculation contradicted the direct observation of the child's pediatrician. Significantly, this court-appointed evaluator appeared to consider the creation of such symptoms in the child an important element of the MSBP diagnosis. The same expert who labeled the mother as a case of MSBP because of her sex abuse allegations curiously, however, admitted she did not have the basic knowledge or expertise to assess whether the mother had overused the health care system to treat her daughter's spina bifida condition. In her written report to the court, she explained that in order to determine this a "full review of the medical records . . . would need to be done by two medical doctors, a pediatric specialist and a psychiatrist." In her testimony, she likewise told the court that she rested her diagnosis of MSBP on the sex abuse allegations, since she did not "feel comfortable with the medical aspects" of the daughter's case to render an MSBP diagnosis based on those.⁴⁸

Thus, the mother was diagnosed as having a mental illness that forced her to seek constant attention from health care professionals through the manufacture of symptoms—and yet no one ever determined that the mother had induced or exaggerated any of her daughter's medical symptoms, even though her condition gave the mother ample opportunity to do so.

The psychoanalyst explained this peculiar application of MSBP by itemizing ten of the mother's personality characteristics that she considered consistent with MSBP: in particular, the mother was "a pathological liar," had "a great need for adoration," was "manipulative and charming," and was "obsessed with the child's illnesses, among other things."⁴⁹

Yet, *even in support of her sex abuse allegations*, the mother had never taken her daughter for an invasive medical exam, nor was she accused of having her daughter undergo any unnecessary treatment for abuse, such as extensive psychological counseling—nearly always a principal symptom of MSBP.

Despite the shaky evidence of mental illness in this case, the court-appointed expert did not hesitate to recommend the extremely harsh step of transferring the child's custody immediately to the father. In fact, she went further, recommending that the mother be limited to strictly supervised visits. To support this step, she evidently thought her diagnosis insufficient; she invoked instead the all-too-common specter of parental "mutiny," telling the court the mother should have *only* strictly supervised visitation to "prevent her from taking the children [sic] from the jurisdiction or possibly doing damage to herself or the child."⁵⁰ It is not clear what was supposed to give rise to this concern: the mother had never attempted to take the child out of the court's jurisdiction and had never done any harm to the child. Moreover, as we have seen, MSBP was never clearly applicable to this case in the first place. But this was family court. The mother had made a sex abuse allegation; the allegation had not been proved; therefore, the mother required punishment. The mental diagnosis was needed only to supply a cog in a well-oiled machine.

MENTAL ILLNESS THAT GROWS IN A VACUUM

Another suspicious feature of family court mental health diagnoses is that many of the alleged conditions appear to grow in a vacuum. None of the mothers we studied who were given mental illness labels in family court had any prior history of mental illness. None had any history of substance abuse or sociopathic behavior. In fact, many of these mothers admittedly functioned well in their jobs and in their personal lives (serv-

ing as Little League coaches, active in PTAs, taking a leadership role in their churches and synagogues). They became “mentally ill” only when they accused their children’s fathers with acts of abuse.

But, according to most experts, this is not the way mental illnesses work. True, sudden trauma can cause certain short-term, episodic mental illnesses, such as “situational adjustment disorders” or “reactive clinical depressions”—but these are very different from the “psychosis” that family court experts claim to find in protective mothers. What is more, these mental illness symptoms are usually evident over a significant period of time and affect the person’s daily activities. In fact, at a conference cosponsored in 1999 by the National Alliance for the Mentally Ill on the effects of mental illness on employment, it was agreed that 85 percent of people with serious mental illnesses are unemployed—or, if employed, are working at jobs that are “poorly paid and sporadic.”⁵¹ By contrast, protective mothers, supposedly too “ill” to care for their children, are in most cases gainfully employed and reasonably well paid. Thus, it is very unlikely that so many of them can in fact be “mentally ill.”

Only in family court does “mental illness” mean something sharply different from what most people, professionals and others, understand it to mean. An aide to a state senator, concerning the case of a certain protective mother, told one of us, “I just know that mother is not mentally ill in spite of what the court says. I just know [because] she’s so easy to deal with.” In fact, this mother was “mentally ill” only because she believed eyewitness testimony that her daughter had been abused.

MENTAL ILLNESS THAT REQUIRES HARMFUL “TREATMENT”

Another damning feature of these theories is that the “treatment” recommended for the “illnesses” they propound is exactly what is worst in cases of actual child abuse.

What do the believers in these theories recommend? Isolation of the child from anyone who believes in the abuse the child is reporting; supervision of the child’s contact with the protective parent, so that the child is discouraged from describing abuse; forcing the child into the custody of the very parent he or she fears. A better prescription for intimidating genuinely abused children, forcing them to hide their painful truth while punishing the parent who has tried to protect them (and incidentally rewarding the abuser), can hardly be imagined.

Richard Gardner, for example, wrote emphatically that “the most potent therapeutic measure that one can utilize for PAS children is the reduction of their access to the alienating parent,” claiming “there was a significant reduction or even elimination of PAS symptomatology in all . . . of these cases.”⁵²

Significantly, Gardner reached this radical conclusion—with appalling implications for any parent who suspects abuse, and for any child who needs a parent’s belief and protection—after just one telephone interview (lasting no longer than twenty to thirty minutes) with each of several parents who had been accused of sexual abuse and now had full custody of the allegedly abused child. According to Gardner, these parents claimed that PAS completely disappeared after custody was awarded to them, particularly when their child’s visits with the other (formerly accusing) parent were limited. (How Gardner could have expected the alleged abusers to report anything different from this is not explained.) Since no other researcher ever interviewed these parents and since Gardner did not even attempt to use objective criteria that could be reviewed by other scientists, it is hard to see why these self-serving statements from some accused parents deserve any professional consideration at all. But

Gardner was convinced—he wrote without hesitation: “This study [sic] provides confirmation of my longstanding observation that the most potent therapeutic measure that one can utilize for PAS children is reduction of their access to the alienating parent.”⁵³ He might have added: It is certainly the most potent measure for ensuring that a child who has reported sex abuse by a father will never report it again.

Psychiatrist Johnathan Gould (whose work was mentioned earlier in this chapter) clearly had such radical proposals in mind when he cautioned that “experts have often been criticized for overstepping the bounds of their expertise, providing opinions of a personal nature couched in the aura of empirical science.”⁵⁴ Gardner’s methods do even more than that, since on the basis of his personal opinions he was willing to recommend steps certain to be devastating to children who have actually been abused. The same is true of approaches similar to Gardner’s. Only in family courts do such theories flourish.

AN UNHEALTHY SYMBIOSIS BETWEEN MENTAL HEALTH PROFESSIONALS AND JUDGES

The theories we have been examining are not only questionable as mental health diagnoses. Used in family courts, they have the dangerous effect of blurring the line between judge and psychologist. The fact-finding process in an abuse case, which is supposed to be the responsibility of the judge (abuse cases in family courts are almost never tried before a jury), is subverted by a mental health expert who claims to be able to determine that a mother’s personality makes her accusation incredible. Similarly, judges are encouraged by the use of such theories to play the role of therapist, prescribing remedies meant to “treat” a mother while in fact they involve the court in abuses of its judicial power. This unhealthy symbiosis corrupts the legal process. It also helps to explain some of the stranger aspects of custody-related abuse litigation.

Doctors Play “Judge”

It is a fundamental principle of family court jurisprudence that courts may not delegate their fact-finding authority to anyone, including evaluators or mental health practitioners, in any matter pertaining to the welfare of a child.⁵⁵

However, as we have seen, some of the mental health theories used in family courts have precisely the effect of short-circuiting the judicial function. This is particularly likely—and particularly wrong—when a purported mental health theory places great stress on the legal context of a sex abuse allegation. Such theories seem tailor-made to allow an evaluator to judge the fundamental question in a sex abuse case based on his own (nonprofessional) prejudices. It comes as no surprise that some of the experts whose approaches we are examining here seem convinced that if a sex abuse allegation is raised in the context of divorce and custody it *must* be false—even though this contradicts much of the published research. When such experts perform evaluations of accusing mothers, they tend to base their diagnostic conclusions on “legal” factors—the presence of a pending divorce/custody action, or the posture taken toward the case by a family court judge—rather than on the clinical features of the mother’s personality functioning, although the latter are the only things the evaluators are professionally competent to evaluate.

A single case illustrates this point. A Brooklyn, New York, protective mother was accused by CPS of “brainwashing” her daughter to believe she had been sexually abused by the child’s father (even though an eyewitness had reported the abuse incident). A psychiatrist, who was board certified in child and adult psychiatry and neurology, was asked to evaluate the mother for mental illnesses—and was pointedly told the legal “line” on her case: that she was paranoid, believed she was being conspired against by CPS and family court, and had fabricated the sex abuse charge.

The psychiatrist's report illustrates the correct way to handle an examination for mental illness in such a case. She noted that the mother's allegations of a court conspiracy to cover up sexual abuse seemed difficult to believe. On the other hand, she found no evidence of psychotic disturbances in the mother. She reported: [The mother's] current clinical picture presents a complex diagnostic question. Some of her behavior could be classified as "paranoid" given the themes of being conspired against, maliciously maligned. . . However, she does not present any history of social isolation, seclusiveness or eccentricities of behavior. She does not show restricted affectivity and in fact, can show genuine warmth with interest in the person she is interacting with. . . All in all, you are left with an academically and professionally successful woman, who . . . had been successfully raising her daughter prior to [the time of removal].⁵⁶

The expert also specifically indicated (on Axis II of the "Diagnostic Impression" chart contained in her report) that the mother had no mental illness. Thus, she limited herself to a clinical evaluation, based on the facts available to her, leaving to one side the court auxiliaries' claim that the mother was "delusional." She not only concluded that the mother had no mental illness but urged the court to immediately return the child to her.⁵⁷

This, however, was not the result desired by the law guardian, who had selected the psychiatrist. In fact, the law guardian actually refused to present the expert's report to the court or to call her as a witness.

The family court judge later ordered an evaluation of the mother by the controversial Dr. Arthur Green. Green's evaluation recklessly did what his predecessor had carefully refrained from doing: he mixed the roles of judge and evaluator, using legal proceedings whose propriety he had no way of judging to control his psychiatric diagnosis. First, he told the mother that no amount of evidence would persuade him that the alleged abuse had actually occurred; after all, he said, the judge, the judge's clerk, and others involved in the case had already told him

it did not!⁵⁸ He also told her (at their very first session) that because of what the judge had told him, he already knew that she was “paranoid” and “delusional.” He thus effectively made his diagnosis before even examining his client. (Interestingly, Green claimed he was able to reach his conclusions about the alleged abuse because the “finding” that it had not occurred, and that she had invented the charge, was “the law of the case.” He thus not only invoked a legal doctrine to justify a psychiatric diagnosis—he *misapplied* the doctrine, since no such formal findings could have been made while the trial was still proceeding. Green, a nonlawyer, clearly did not understand this.)⁵⁹

Throughout his report to the court, Green referred to the mother’s attitude toward the case against her as the basis for his diagnosis of “paranoia,” writing: “She [the mother] expresses an abundance of projective and paranoid mechanisms. She maintains that there is a conspiracy against her. . .” In fact, Green noted specifically that “[a]side from [the mother’s] delusional beliefs concerning the judges, [agencies,] and myself, there is no evidence of disorganized thinking, or hallucinations, or other behavior which might be associated with a Schizophrenic disorder.” Thus, he based his diagnosis of paranoia squarely on issues he could not, as a clinician, legitimately resolve.⁶⁰ In this respect his approach was directly opposed to that of the former evaluator, who based her diagnostic impression on her own clinical observations rather than the “running record” built up by hostile court officials and law guardian and social service agencies.

Green’s evaluation formed the basis of the family court’s decision in the case. Yet it was clearly flawed as a psychiatric diagnosis, not only because it assumed the existence of a “circumscribed psychosis” (a peculiar thing indeed), but also because Green relied, for his diagnosis, on determinations by judges and agency lawyers that he, as a mental health professional, could not evaluate (and that were, as we have seen, legally premature). As a result of Green’s evaluation, the mother lost

custody. She eventually lost all visitation, too, including the right to write letters and make telephone calls to her daughter.

Another problem caused when the evaluator takes over the judge's role—a problem virtually unique to family courts—is that an evaluator may take it upon himself to *predict* legally significant acts, such as a mother's defiance of a court order, before they occur. This can have fateful consequences. We have already discussed a case in which a mother was limited to supervised visitation because a mental health expert claimed that her "illness" might cause her to kidnap the child. This is unfortunately not rare in family courts. It is not unusual for family courts to impose restrictions on mothers who have never violated a court order, simply because a psychologist has claimed that she conforms to some sort of a psychological "profile" of a runner. Experts have made such assessments based on brief interviews, and have made them whether or not the mothers have stated any plans to go "underground."⁶¹

"Guilty" by Reason of Insanity

Another unhealthy feature of the mental health theories we are examining is that they encourage evaluators—and judges—to eliminate one of the elements of proof required by the law before a protective mother can be punished. In this way, these theories actually erode a protective mother's legal rights.

This occurs when mental health evaluators claim that a mother's sex abuse accusations, although not malicious, must be treated as harshly as if they were, because they are assumed to be the products of mental illness—and therefore will continue unless restrained. The legal significance of this is worth a bit of explanation.

Under ordinary circumstances, a mother who makes an accusation of sex abuse—even if that accusation turns out to be impossible to prove

or legally unfounded—cannot be punished for doing so unless her accusation was intentionally false. If she acts maliciously against a child's father in deliberately inventing a charge against him, she is alienating the child from the father and is subject to penalties for doing so. Such penalties may include loss of custody, if a court concludes that this is the only way to prevent her from turning the child against his or her father.

But if she is simply mistaken in her suspicions, no penalty may be legally applied to her. We have previously mentioned (see Chapter 4, note 15) a New Jersey appellate court that ruled a family court cannot punish a mother for her subjective beliefs about a child's sexual abuse, even if the court does not ultimately endorse them. This ruling reflects fundamental legal principles.

But this, of course, assumes that beliefs are not automatically translated into action. That is, a woman who knows her belief that sexual abuse has occurred was not ratified by a court can refrain from alienating the child from the father once she knows she is legally forbidden to do so. This is the assumption the law makes of all ordinary people. It must do so; otherwise, a great many people could be treated as guilty until proved innocent.

But exactly this assumption is challenged when a mother's belief that sexual abuse has occurred is translated by a mental health theorist into a product of mental illness. The mentally ill do not enjoy the benefit of the assumption of a distinction between belief and action. Illness is not controllable; the woman who suffers from it will continue to act, uncontrollably, no matter how forcefully she is told not to. If she believes, "pathologically," that a man has sexually abused her child, she will continue to intervene between that man and that child no matter how many times a court rules against her. There is no way to prevent this—unless the court deprives her of the opportunity by removing the child from her custody, supervising her visits, and so forth.

Therefore, the woman whose accusations are said to result from PAS or MSBP will be treated just as harshly as if she were actively fabricating the accusations that, in fact, she is making sincerely. This gives family courts a dangerously appealing excuse to treat protective mothers as guilty until proved innocent. If they are said to be mentally ill, they are automatically malicious—or, rather, they must automatically be treated as if they were, because their thoughts supposedly spill over into action. In truth, actual malice is difficult to prove. Sex abuse charges are rarely exposed as intentional fabrications. But the substitution of “mental illness” for malice in the lexicon of family court quackery allows courts to ignore the need to prove guilt—and to punish protective mothers without the same degree of proof they demand before punishing accused fathers. In short, these theories serve as legal maneuvers that circumvent the burden of proof that would otherwise be required to stigmatize an accusing mother, and, as a result, they can turn an entire sex abuse case legally upside down.

Judges Play Doctor

While family court evaluators play judge, family court judges often find it convenient to appoint themselves mental health experts in order to couch their criticisms of mothers in what looks like “expert” language. When, for instance, a judge tells a custody evaluator that the mother is “crazy,” the judge acts as the arbiter of her sanity, when he should defer to the clinician to make such a diagnosis.

A Texas judge, in 2001, played doctor when he removed a two-year-old girl from her mother’s custody on the strength of his own “diagnosis.” An entry in the judge’s handwritten log on the case, made a few days before he removed the child, used the phrase “parental alienation.” Yet the custody evaluation later written by the court-appointed expert contained no reference to parental alienation of any kind.⁶²

In a Brooklyn, New York, case, both the CPS case supervisor and a CPS psychiatrist expressed concern that an allegedly abused six-year-old girl showed “sexual provocativeness” during interviews, spreading her legs in front of both men and, according to the psychiatrist, watching for his reaction when she did so. Both professionals considered this a possible symptom of sexual abuse.⁶³ The judge, however, evidently determined that there should be no evidence of sexual abuse in the case, decided to substitute his judgment for the psychiatrist’s: he had seen the same movements when he interviewed the child privately in chambers, he wrote in his decision, and “I found this to be not at all sexually provocative, but merely the nervous movements of a child too frequently interviewed.” Nor was that all. A psychiatrist retained by the child’s law guardian had concluded that the mother had no mental illness; the judge impatiently substituted his own conclusion for the expert’s, claiming (contrary to the psychiatrist’s own findings) that her “report reveals a seriously disturbed person.”⁶⁴ Another psychiatrist found no evidence that the mother was mentally ill; the judge concluded that the psychiatrist “really does not understand the nature of the proceedings here in Court.”⁶⁵ Yet another mental health expert, after administering a full battery of personality tests to the mother, concluded during “lengthy” testimony that the mother would be a proper custodial parent; the judge dismissed this expert conclusion as “quite plainly wrong.”⁶⁶ He even offered his own “diagnosis” of the mother’s courtroom behavior, claiming she was “histrionic and grandiose” while testifying.⁶⁷

Unfortunately, playing doctor in this fashion is not at all uncommon in family court. In one New Jersey case, the mother was subjected to three separate court-ordered psychiatric evaluations over a nine-month period. Not one of these psychiatrists made a diagnosis that she was mentally ill or unfit. In fact, all three recommended that she have custody of her three children.⁶⁸ Nevertheless, the family court judge did not hesitate to refer to the mother’s “illness” in stating his findings.⁶⁹ He

awarded custody to the father and found the mother guilty of “emotional neglect.”⁷⁰

Unhealthy Consequences of the Symbiosis

It can clearly be seen that the use of family court quackery corrupts the legal process by short-circuiting the proper legal inquiry into whether an alleged act of abuse actually occurred or not. This results directly from the nature of the theories themselves, which encourage psychologists to dodge the factual complexities of an abuse case and “resolve” it without evidence.

Dr. Richard Gardner provides a very clear example of the way in which psychological “evaluators” have crossed over into a judge’s role on the strength of questionable theories. In fact, Gardner’s published work proves again and again that he could not offer an evaluation of Parental Alienation Syndrome (as he did in hundreds of cases) without reaching a conclusion that no abuse had actually occurred. Although he generated a very detailed list of symptoms to describe PAS (e.g., poor bonding with the accused parent, overt hostility toward the accused parent that can spread to the alienated parent’s extended family, and weak or absurd rationalizations for the deprecation of the accused parent), he claimed that *all* these symptoms can be disregarded if the abuse really occurred.⁷¹ In a 1998 article he was equally emphatic: “I have repeatedly stated that when true abuse is present, then the PAS [Parental Alienation Syndrome] diagnosis is not applicable in that the children’s animosity is justified.”⁷² Yet Gardner never explained on what basis he was able to reach a conclusion as to whether or not abuse actually occurred. One must conclude that every one of his diagnoses of PAS included a factual determination that no clinician in Gardner’s position can be competent to make (and that Gardner himself never justified).

Indeed, Gardner seemed to don a judge’s hat in family court litigation more often than he wore the hat of a clinician. In 1998, Professor

Faller (mentioned above) carefully critiqued a large body of Gardner's work. She concluded that he "does not provide any research findings to substantiate his assertions about proposed characteristics and dynamics of the Parental Alienation Syndrome."⁷³ In fact, Faller pointed out that Gardner's book *Sex Abuse Hysteria: The Salem Witch Trials Revisited* (1991) dismissed the idea of "proof" in clinical matters: "The term scientific proof is not applicable to most of the issues discussed here. . ." Gardner, she wrote, even "goes on to refer to the standard and accepted practice of citing support for professional opinions in existing literature as 'specious buttressing.'"⁷⁴ If that is true, however, then on what "proof" were so many mothers condemned as suffering from PAS, with the result that their allegations of abuse were not only ignored but were turned against them? How did Gardner "prove" that their children had not actually been abused? When a clinician wears a judge's hat, even the law can be rewritten by a psychologist. In one case, a clinician actually took it upon himself to rewrite the legal definition of what constitutes child sexual abuse.

This occurred in a case in Buffalo, New York, that arose in the mid-1980s. A court-appointed expert testified as to his understanding of what constitutes child abuse:

LAWYER FOR MOTHER: If a seven year old *consented* to sexual touching, then that would not be sexual abuse in your definition?

PSYCHOLOGIST: I would say that's inappropriate. I would not call that sexual abuse.⁷⁵

In fact, under New York law, an adult is *guilty of first degree sexual abuse*—a felony—whenever "he or she subjects another person to sexual contact" when that other person is less than eleven years old; the law further defines "sexual contact" as "any touching of the sexual or other intimate parts of a person not married to the actor," including "the touching of the actor by the victim, as well as the touching of the victim by the actor."⁷⁶ In this case, the father had allegedly touched the girl's

genitals and placed her hand on his genitals. Since she was only seven, these acts clearly constituted child abuse under the explicit language of the law. (In fact, they constituted a felony.) Thus, the expert in this case actually overruled state law in positing his own definition of sexual abuse! The judge, however, accepted the expert's definition over the statutory one and awarded the father accused of sexually abusing his daughter full custody of his seven-year-old child.⁷⁷

"PARENTAL ALIENATORS"

This section shows, in more detail, how these theories work in actual family court cases. What follows are the stories of three different women diagnosed by court-appointed experts as "parental alienators" because they supposedly fabricated charges of sex abuse in an effort to turn their children against their fathers.

A Sex Abuse Allegation as Proof of the Mother's "Fantasy" In 2000, a Georgia protective mother was suddenly called into court for an emergency hearing (held at the law guardian's request) in which she lost custody of two young girls, aged six and eight, that very day. This emergency hearing was prompted by a court-appointed mental health expert's report on the mother, written the day before. According to the report, the mother suffered from "impaired reality testing and tendency to use fantasy to excess."⁷⁸ In interpreting the psychological tests given to the mother, the expert made the following claim:

tends to use fantasy excessively. The Rorschach findings indicate that in situations of stress she is likely to "defensively" substitute fantasy for reality. This is a form of denial that provides some temporary relief in that it may replace an unpleasant situation with an unrealistic but more manageable situation. This tendency usually results in a dependency on others

to solve problems. In other words, people that do this often think that external forces will resolve a situation if they can avoid the problem long enough. This is a particularly destructive stylistic tendency for [the mother] given her problems in thinking. This increases the likelihood that her fantasies will involve considerable distortion of reality.⁷⁹ (This mother)

None of this appeared to suggest an “emergency”—but at the hearing the next day, the expert played her trump card, claiming that the mother “is implanting false memories [of sexual abuse] and she is alienating both of these children from their father.”

Then, putting on a lawyer’s hat—situatedly achieving her role as “therapist turned lawyer”—she used the occasion to advise the court concerning its proper legal course. She told the judge, “I don’t believe anything short of a *dramatic intervention by the court*—and even that might not be effective—is going to have any impact on this [the children’s hostility toward the father].”⁸⁰

Significantly, the expert disregarded the children’s statements to two other psychologists, one of whom had made an oral report to the state’s child abuse hotline that the father had “fondled” the children while they were in bed at night.⁸¹

These two psychologists had interviewed the children a number of times and concluded that “the children did not appear coached” and, indeed, showed the proper “affect associated with this disclosure [of sexual abuse].” They had also found that the older child “appeared distressed in a way that would be consistent with that kind of [sexual abuse] disclosure.”⁸² Nevertheless, the court’s expert was not only convinced the abuse was a “fantasy” of the mother but was prepared to demand the isolation of the girls from their mother.

Louisiana attorney and national child abuse advocate Richard Ducote, in a memorandum on this case addressed to the Cobb County Bar Association and to the Metro Atlanta Board of Mental Health Professionals,

alleged that the expert had been misled by the child's law guardian, her "good friend," who had "'assured' her that the sexual abuse allegations were false."⁸³ Once led to believe (through this unofficial persuasion) that the charges could be ignored, the evaluator found many of the criteria of Richard Gardner's Parental Alienation Syndrome present in the two girls. In her report claiming an "emergency," she noted "the following features of parental alienation":

1. a campaign of denigration against [the father];
2. weak, frivolous or absurd reasons were given for the deprecating feelings and statements;
3. a lack of normal ambivalence toward the parents (all good and all bad);
4. child's opinions are independently initiated [in one child only];
5. automatic support for the favored parent;
6. almost complete lack of empathy for the feelings of the alienated parent; and
7. borrowed scenarios or "adult wording."

But had the expert not assumed the falsity of the allegations, these same symptoms could have been very differently construed. Similarly, had the expert not assumed the allegations were false, the mother would not have been diagnosed with "impaired reality testing" and a "tendency to use fantasy."

Besides basing her entire diagnosis on an assumption about the falsity of sex abuse allegations (reached *unofficially*, not after a judicial inquiry), the expert abused her clinical role by telling the court, in her report, that the mother "has discussed with the Examiner taking the children and going underground."⁸⁴

As we have seen, such a claim is almost always designed to enrage a family court judge. It encourages the court to treat the mother as if she

were in violation of court orders and guilty of parental kidnapping before she has done any such thing. We have also seen that this is a corruption of the role of a mental health evaluator, who cannot properly predict on psychological grounds that a protective parent—simply because she is a protective parent—will defy the law. In this case the claim was doubly cynical, because it was virtually an invention: when the evaluator was cross-examined by the mother’s lawyer, she admitted she could not recall when the mother, who had been seen four times over the course of four months, had made such a statement. Nor could the expert recall any specific details. In fact, she finally conceded that the mother’s supposed mention of the underground did not cause her to believe it was “serious enough or compelling enough” to be an “emergency.”⁸⁵ But that was precisely what the evaluator had claimed—when she needed a reason to have the children removed from their mother.⁸⁶ She had even recommended harsh specific measures to be taken to prevent a parental kidnapping: “Additionally, it would be imperative to notify both girls’ school that their mother was not allowed to visit [and] not to take them from school without expressed written permission by [the father].”⁸⁷

The unhealthy combination of a legal prejudgment and an accusation of future lawbreaking, masquerading as psychology, had the desired result. The abuse charges were never fully investigated, despite the substantial evidence supporting them. The children were immediately removed from their mother as a result of the “emergency” claimed by the evaluator, even though (as we have seen) no real emergency could have existed. This mother never regained custody of her two daughters. In fact, she has not been allowed to see them for over four years.

A Mother Forbidden to Express Love to Her Children

In 1995, a California mother who had been divorced for about a year became another woman accused of being a “parental alienator.” Several months later, she lost custody of all three of her children (a boy, aged

ten, and two girls, aged four and seven) to her ex-husband. In addition, she was cut off from any contact with the children for almost half a year; thereafter she was limited to strictly supervised visitation for another four years. The court battle, which began several months before the mother lost custody of her children, was precipitated by the girls' disclosures of sexual abuse. The parents were sharing joint custody, with the children spending weekends with their father. When the girls told their mother they had been sexually abused at their father's home, the mother asked the family court to suspend the father's visitation until the charges could be fully investigated.

The mother's concern that her children were being abused was supported by several facts. First, both of the children's therapists strongly suspected sexual abuse. The four-year-old's therapist reported that she showed features "in her spontaneous play that are consistent with a child who has experienced trauma."⁸⁸ The other girl's therapist reported, in a sworn statement, that when asked to write (symbolic) letters to her father the child wrote: "I wish you would stop touching me in my privates. Stop doing that because it hurts! Why didn't you stop when I told you to stop?"⁸⁹

Second, a medical examination of the older girl showed genital abnormalities that raised the question of sexual abuse. The girl had been examined with a colposcope that showed "a narrow irregular thickened hymen," and "generalized increased vascularity" of the perihymenal tissue.⁹⁰

Third, the older girl was interviewed by the Multidisciplinary Interview Center (MDIC), a team of forensic scientists used by the local district attorney's office. The girl had told them: "He [the father] pulled down my pants . . . put his finger in there [pointing to her genitals] and then he pulled up my pants and put his 'wee-wee' in my mouth."⁹¹ This tape-recorded interview formed the basis of a police report against the father.⁹²

Yet, despite this evidence, the case turned sharply against the mother once the court appointed a mental health expert to assess the validity of the sexual abuse claims. After conducting six interviews with the mother and her children, he wrote in his report: "I also find the mother utilizing numerous exclusionary maneuvers and she is presenting the father as inconsequential and unimportant [to the children]." ⁹³ His testimony, in which he invoked Gardner's PAS theory, was harsh and clear:

MENTAL HEALTH EXPERT: My recommendation is that all the children should be placed immediately in the father's custody, and that the mother should have absolutely no contact with these children for thirty days. father's counsel: You are basing that on what concept that you are aware of?

MENTAL HEALTH EXPERT: Well, this is part of this Parental Alienation Syndrome and my observations and my opinion is that these children do not hate their father and that's one of the prime symptoms of Parental Alienation Syndrome is that the children hate their father. Children are not born with the gene to hate their father. So I see that all this alienation that's been going on by the mother has not worked. The father has been persistent in trying to get contact with these children, when it hasn't worked, now we have the atom bomb. This is right out of textbooks.

FATHER'S COUNSEL: So now you are saying since what other maneuvers mother might have gone through, the newest one is the allegation of sexual abuse [which] is just a new twist on the alienation syndrome?

MENTAL HEALTH EXPERT: This is an example of what Dr. Gardner calls escalating of more bizarre, more unusual allegations to try to get the father out of the children's lives and when these don't work, they get stranger and more bizarre. Then they still don't work, then this is one sure way to get a father out of the children's lives absolutely. ⁹⁴

The expert was convinced that this was a clear-cut case of Gardner's Parental Alienation Syndrome; he told the court that it "matches right out of the textbooks what's classic in these Parental Alienation Syndromes. Just makes me even more confident that the other allegations haven't worked and here's the atom bomb."⁹⁵

The expert was so confident in this diagnosis that he dismissed the concerns of the children's therapists: "Little weight should be placed on the impressionism [*sic*] and anecdotal intuitive statements made by the children's therapists."⁹⁶ In his testimony he was just as dismissive of the children's own statements:

I don't place a lot of weight on what children have to say. I listen to them, but I don't place a lot of weight on what children have to say. Otherwise I would ask children which stocks and bonds I should buy, how should I run my life, I don't listen to what children say.⁹⁷ Armed with a diagnosis of PAS, the court followed Gardner's recommendations practically to the letter. As we have seen, Gardner routinely recommended limiting the allegedly abused children's access to the parent who believed their report. In keeping with this approach, the court suspended the mother's visits with the children.

After almost six months of complete isolation, the court ordered supervised visitation between the mother and her children at a special clinic run by a program that operated through the court. The clinic closely monitored the mother's visits with her children. Naturally, the clinic's personnel were quickly introduced to the case's "running record"; its director later testified that the clinic was convinced the mother was guilty of PAS because of "information . . . received thus far."⁹⁸ There is no evidence that this "information" included any of the evidence supporting the mother's suspicions of abuse or the concerns of the children's therapists.

It was not enough for the mother to be constantly monitored during her short visits with her children. Soon, the program director of the monitoring clinic wrote to the judge to urge that the mother be forced to undergo therapy with her, the director, rather than with the therapist the mother was already seeing. To justify this demand, the director argued that “it would be necessary for the therapist to understand that [the mother’s] treatment needed to be oriented towards her alienating behavior and not treat her as if she were a non-offending parent of molested children.”⁹⁹ After all, she complained, the mother’s current therapist was “reinforcing her image of herself as a parent of molested children.”¹⁰⁰ The court agreed.

Now the mother lived in a closed world of mental health insanity. She was not supposed to consult a therapist she trusted. She was not allowed to speak to her children about the abuse they themselves reported. She was required to admit that she was mentally ill as a condition of being allowed to see or speak to her children at all. Every aspect of her life as a mother—as an ongoing practical accomplishment—was now controlled by “experts” who, in turn, were guided by the theory of the mother as the “parental alienator.”

Under the experts’ influence, the family court took some bizarre steps. For example, at the time the judge stripped the mother of custody of the three children, he ordered her to have “no telephone contact” for a period of thirty days. At the expiration of that period, the judge ordered that all “telephone contact shall be monitored by an independent supervising adult or shall be recorded for later review.”¹⁰¹ Within a month, the program director, who—along with the children’s father—was intently listening in on every conversation between the mother and her children, asked the court to suspend all telephone contact once again. Her reason? The mother was telling the children she loved them.

In an affidavit written in response to this “expert” attack on the mother’s conversations, the mother’s therapist had this to say: [The mother] was not told that she could not verbally express love for her children over the

phone, as any normal parent does, without the children first expressing their love to her. I must take issue with the statements from the program director that [the mother] always expressed her love to the children first—that is not so. There are several places in the phone recordings that the children express their desire to visit with her and their love for her first, the most poignant of which is the first call between [the mother] and [the older daughter] when [the older daughter] broke down sobbing for several minutes.

The program director criticizes [the mother] for talking with the children about their daily activities, their schooling, their playmates, and world news. At no time in any of the tape recorded phone conversations did [the mother] ask the children how their father was treating them, or make any suggestions about how they should or should not react around their father. If [the mother] is precluded from talking with the children about their normal daily activities, what is left to talk about?

It appears from the entire tone of the letter from [the program director] that no matter what [the mother] said or did, [the program director] could find fault with, doubt the wisdom of, or second-guess the motivation of [the mother] and her ordinary, everyday conversations with her children.¹⁰²

Notwithstanding this protest, the court took the program director's advice, and—because the mother had told the children she loved them—suspended all telephone contact between this mother and her children for four years.

The Mother Who Told Her Daughter She Missed Her

In 1991, a New York mother, who had previously stipulated to joint custody with her ex-husband of their five-year-old daughter, started to worry about what was happening to the girl when she visited her father at his new home in Georgia. According to the mother, the girl returned

from visits with her father in a belligerent mood and began to “act out” sexually. Concerned about the child’s behavior, the mother took her to a psychologist. The child reportedly complained to the psychologist that her father made her sleep in the same bed with him and his girlfriend and that he fondled her genitals during the night.

The psychologist suggested to the mother that the child be evaluated by a clinic specializing in child sexual abuse trauma and referred the mother to a pediatric nurse who headed a sexual abuse program at the clinic. The nurse interviewed the girl and found that she had been sexually abused and that her father was the likely perpetrator. As the nurse later wrote to the family court:

In my professional experience, I’ve evaluated over two thousand children for child abuse. I was a member of the Committee on Sexual Abuse, Office of Projects Department, Supreme Court of the State of New York, Appellate Division, First Department. I co-authored the chapter “Medical Protocol for Evaluation of Sexual Abuse” in the book published by this committee. I am confident about my assessment of [the child] that she is a sexually abused child who states her abuser was her father.¹⁰³

But when the mother asked the court to restrict the father’s visitation based on the evidence of sexual abuse, she was compelled to undergo an evaluation by a court-appointed mental health expert. After interviewing the child and her parents, the “expert” did not hesitate to label the *mother* as the source of all the trouble. According to the “expert,” the real danger involved the threat the mother posed to the child’s relationship with her father.

In her report to the court, the expert not only made the father-daughter relationship her central concern, but did not hesitate to predict that switching the child abruptly to the father’s custody would be good—even necessary—for the child:

[The child] will not, in my opinion, be traumatized if the physical custody is changed so that she [the child] resides in Georgia with the father and [his girlfriend]. It is the only way that she can have any kind of *father-daughter* relationship develop and it has to be done right away before any more damage is done that will be irreparable. One more period of six months [without the father] and I think it will be too late.¹⁰⁴

Besides the abuse reports, this little girl presented some special problems. Since an early age, she had suffered from moderately severe hypoglycemia that required a diet of small, frequent meals.¹⁰⁵ The mother had been instructed by the child's pediatrician to keep a close watch on the child for signs of agitation and restlessness that could signal a precipitous drop in her blood sugar level.

The mother had always carefully followed the pediatrician's instructions. But now that she had been labeled an "alienator," her conscientious behavior became one more weapon against her. The mother was now judged entirely as a problem—an illustration of how mental health experts continually "negotiate" and "interpret" the meaning of maternal "fitness"—so that even her attention to the child's diet was seen as self-aggrandizing and "controlling," as in these comments by the same expert:

came equipped with orange juice, saying that [the child] had hypoglycemia and had to eat something every hour. She insisted she had to know how long the child was to be with me. . . After⁴⁰ minutes a knock came on the door and the mother said she had to come into my office and personally give [the child] orange juice. . . [The child's doctor was] giving the mother the license to use intrusive and controlling maneuvers about food intake with the child. It enabled her to present herself in the role of watchful guardian of the child's health.¹⁰⁶ (The mother)

The expert diagnosed the mother with “Narcissistic Personality Disorder . . . that interferes with her functioning as a mother.”¹⁰⁷ The expert gave the following basis for the diagnosis after administering a battery of personality functioning tests to the mother:¹⁰⁸

The test findings, both the Rorschach and the MMPI, point to a narcissistic character with hysterical traits and paranoid aspects, who feels emotionally unique and different; who has a related sense of elitism and entitlement. Her “found cause” of sexual abuse has enabled her to occupy a very high moral ground and she has found a “righteous cause” that effectively protects her against any dubiousness about her position or claims.¹⁰⁹

The family court obligingly transferred the girl into the father’s custody. Even after those proceedings ended, the findings of the court-appointed expert haunted the mother for many years. When she tried to visit her daughter in Georgia, a judge in that state (after receiving case information from the New York court) gave the father permission to engage a psychologist in Georgia to supervise the mother’s visits and to monitor all her calls to her daughter. In keeping with the bizarre theorizing that had landed the girl with an allegedly abusive father in the first place, this new psychologist cautioned the mother to “talk less about your love and the fact that you miss her tremendously and talk more about your own personal life and your own day to day activities.”¹¹⁰ The child was only six years old at the time; her mother, like most mothers of children that age, tended to focus conversation on the child rather than on herself. But normal parental behavior was suspect in the strange world of family court quackery.

As time went on, the father made the child less and less available for visits. The mother made as many as thirty trips from New York to see the child in Georgia only to find that the child was not at home. The mother asked the court to enforce her visitation rights, but that proved futile; she encountered endless court delays (including an unexpected

change of judges) and mounting legal costs until she was too financially drained to continue the battle.

Adding one injury to another, the father—even though he continued to interfere with the mother's visits—was allowed by the family court to demand weekly child support from the mother.

For the child's eighth birthday, the father seemed to relent, agreeing to allow the mother to visit the girl on her birthday. When the mother finished the nine-hundred-mile drive and arrived for the anticipated meeting, carrying several birthday presents for the girl she had not seen for a year, she was met by a sheriff and arrested for allegedly failing to pay child support. (Apparently the same court that had ignored her attempts to enforce her visitation rights had acted immediately on her ex-husband's request.) In her daughter's view, she was handcuffed and taken to a women's detention center, where she was placed in a "suffocating" jail cell. She recalls being "doubled over" in pain from severe menstrual cramps and "trembling uncontrollably"; she begged the prison guards for prescription medication that was in her purse, but they would not give it to her. When she was finally released six hours later, her ex-husband refused to allow her to see her daughter.

While visits between the mother and daughter were being stymied, the mother made every attempt to contact her daughter via telephone. Eventually the child did not even take her mother's phone calls. But the mother persisted; she continued to call her daughter twice a day, even though she rarely succeeded in speaking to her. She has continued this routine for well over a decade, although her daughter—now almost eighteen—has become almost a stranger to her. She keeps a written log of her failed attempts to contact the girl, a graphic record of the wedge driven between mother and child by misguided mental health theories and family court madness. In black felt marker at the bottom is the running tally of the times—over eight thousand now—she has tried without success to speak to the daughter she once tried to protect.

THE EFFECTS OF MENTAL HEALTH LABELS

Mothers on the receiving end of the labels we have been examining feel their effects even outside family court. Women who have always been seen as competent people and mothers are now treated as if they were dangerously insane. How are they to explain to friends and neighbors why they no longer live with their children? Why they cannot even meet them without a supervisor present? When mothers are devalued by the family courts, there is nowhere to hide from the labels that have been applied to them. The mentally ill—even when they are not really mentally ill—are not treated as other people are.

For instance, one New York protective mother was found in the 1980s to be “delusional” about her belief in her daughter’s sexual abuse (although substantial evidence supported her belief). The mother publicized her case by appearing on the nationally televised *Geraldo Rivera Show*. But when the show’s producer called the family court judge to get his side of the story, the judge’s law clerk told him that the mother was a “paranoid schizophrenic.” This was not true: even the evaluator who had called the mother “paranoid” about the sexual abuse had not gone so far as to call her a “paranoid schizophrenic.”¹¹¹ But she was to encounter similar experiences time after time, as she appeared on television or testified before legislators.

Such mothers suffer feelings of self-blame, inferiority, worthlessness.¹¹² One Kansas protective mother said, after she had finished a harrowing two-year family court trial in which she was ridiculed by the court and all its auxiliaries as “the crazy woman,” that she emerged feeling she was “lower than dirt.”

ADDENDUM—NEW AND MORE EGREGIOUS RENDITIONS OF THE PROTOTYPIC PARENTAL ALIENATION SYNDROME (PAS)

New Variations of Parental Alienation Syndrome that Serve as a Detriment to Sexually Abused Children

Parental Alienation Syndrome, notwithstanding having been debunked as junk science,¹¹³ may be seen as a hydra that grows two new heads for each one it loses. In recent years, a number of new permutations of PAS can be found in protective mother cases. Such permutations do not come out of thin air, as they clearly have their moorings in Parental Alienation ideology; as a result, they consequently serve the same function as the traditional PAS theory—which is to discredit the child’s allegations of sexual abuse and punish the mother with loss of custody of her child either to foster care or to the parent accused of perpetrating sexual abuse. Some examples of new names for versions of PAS are “Parental Alienation” (PA), “Resist and Refuse Dynamics” (R-RD), or simply “Alienation” (A). These theories were devised primarily to discredit mothers (not fathers) in domestic litigation.

What follows are two case narratives that show how such variations of a deeply flawed concept are unfortunately welcomed in the family courts throughout the country, and undoubtedly to the detriment of protective mothers and their abused children.

A Mother’s Narrative of Seriously Disturbing Sexual Boundary Issues—and How “Enmeshment” Theories Swept It Under the Rug

In early 2022, the court-appointed custody evaluator in a North Carolina divorce case gave both parents of a seven-year-old girl a lengthy questionnaire. A few of the questions on this standard form related to

child sexual abuse. The mother, who had already unsuccessfully sought the help of the Department of Social Services regarding the abuse of her daughter, and who had already reported her concerns to the court, gave a detailed narration of the troubling sex abuse incidents in this custody questionnaire. The mother reported disturbingly graphic information, which the father had previously admitted when deposed by the mother's counsel,¹¹⁴ although he (the father) downplayed the significance of his actions insisting that the child had initiated these encounters: There were repeated instances of sexual exploitation of [the child] by [the father]. . In 2018, I witnessed two incidents, where twice I walked into [the father's] room and his penis was in [the child's] mouth. [The father] downplayed the events and explained it away as "curiosity" on her part, said it was nothing and indicated that I was overly concerned. I responded by making major changes, including moving [the child's] sleeping arrangements away from him, enrolling her in a school program (though we went back to homeschooling), and not leaving her alone with him in a room or anywhere without my full awareness of what they were doing. I later spoke with him about what I witnessed directly, and though he told me not to make a big deal out of it and that she was "just curious," he agreed to no longer ever be naked around her or to "allow" or encourage her to touch his private areas, and would remain clothed with her at all times.¹¹⁵

Later in the questionnaire the mother was asked to provide historical information about the father's interactions with the child. The mother stated:

When [the child] was a baby, [the father] would sniff her vagina, putting his nose right up to it; I asked him what he was doing—he was dismissive to me and did not stop; I told him it was extremely weird and made me uncomfortable.¹¹⁶ The mother elaborated on other troubling incidents as well. It must be noted that we are sensitive to the graphic nature of the mother's description, which we reproduce below; however, we have decided

not to “sanitize” the mother’s language (which would be quoted verbatim in a court order 10 months later¹⁷). Our purpose is to demonstrate directly from the documents contained in a court record how protective mothers are discredited through the use of bogus mental health labels, and how this leaves children vulnerable to serious boundary violations.

The mother reported:

When [the child] was a baby/toddler, on 3 or more occasions, there was a “family bath” with Mom, Dad, and baby together (started as a bath with mom and baby, then [the father] would come in and ask to join). On one of these later occasions, [the father] was laying down while [the child] and I were sitting at the foot of the bath. He began playing with his penis, calling it “Mr. P,” putting his fingers on either side of it to move it around and make it “talk.” I felt uncomfortable but didn’t immediately say anything. Then [the child] began to play with it. He said (as a disgusting joke), “You want some Daddy milk?” I ignored him and moved her away and said to her we don’t do that, that’s a private space. [The father] immediately “corrected” me with a lecture about how we should not “shame” anyone’s body, and in general must hold a natural, non-shaming attitude regarding bodies, rather than a negative viewpoint.

On another occasion, [the child], then a toddler, and [father] were taking a bath together. I was doing chores around the house, and walking in and out of the bathroom to help with her bath time. I heard him say, “Yep, that’s my penis!” I walked in and saw [child] playing with [father’s] penis and saw her put her face in the water to put it in her mouth. Alarmed, I avoided him but spoke directly to her, stopping it and expressing what a private area is. From that point on, I was uncomfortable with

[father] bathing with our daughter, and took complete responsibility for bathing her myself.¹¹⁸

When the family court granted overnight visitation to the father—who had already separated from his wife several months prior—the mother, in recounting that event, told us she spoke calmly to her husband about the need for “appropriate boundaries” to be upheld at all times so that their daughter would be protected. The mother stated that the last time she had that discussion with her husband was in January 2020, but that her husband “avoided eye contact” with her.¹¹⁹ The mother was reluctant in the years that followed to speak to the child’s father about these concerns for fear that the court would think she was “obsessed” with these issues and trying to place the father in a negative light.

For example, after visits with the father, the child would display disturbing behaviors: namely, emotional regression, night terrors that were hard to placate, and general issues of low self-esteem. Over the next couple of years, the mother noticed two unusual things related to the child’s overnight visits with the father.

First, the father constantly insisted on changing the child’s clothes. Even when picking up the child at the mother’s home, he would often bring a change of clothes for the child and would make the child change in the car—although, given the short distance between the two homes, he could just as easily have changed her after arriving at his place. This baffled the mother so much that she began to wonder whether the father had some ulterior sexual motive (whether he was aware of it or not) for changing the girl’s clothes so frequently. The mother relayed to us that she was astonished by this “routine” because she had been meticulous with her daughter’s hygiene.¹²⁰

Second, the mother learned from a friend that her daughter, then around eight years of age, had told a group of classmates that her father had shown her his private parts when she was younger and encouraged her to engage in “playful” acts. The mother, reluctant to raise

this topic with her daughter since she was now under close scrutiny by the family court personnel, nevertheless discussed this matter with her attorney.¹²¹

Understandably alarmed—especially in light of the father’s admission, however grudging, of sexual “boundary issues” in the past—the mother relayed her concerns to the child’s court-appointed therapist. Amazingly, however, the therapist saw nothing in the mother’s report but evidence that she was unhealthily “enmeshed” with her child. She amplified this “concern” in testimony before the court:

Enmeshment does [happen] when there are unclear boundaries, permeable boundaries between people and, um, there are needs that get met for one person by the other, um, There’s a, there’s an intense need for closeness. Um, that, that kind of gets to the point of problematic there’s attachment, which is a great thing, but then there’s, enmeshment where it’s a little much.¹²²

The expert continued to pontificate about the mother’s condition of “enmeshment” with her child, spinning a broader, long-term view: we’re supposed to give our children roots and wings and in enmeshment you’re kind of overvaluing the roots and not paying attention enough to the wings. Children need to grow away from us eventually, not at seven, but you know, eventually.¹²³

So, whether it’s PAS that is called outright or its derivative, referred to as “enmeshment,” the implications are still the same. The child is, according to these quack theories permeating the family courts, endangered by the mother’s caring and protective response to her child when learning of the sexual violations of the child by the father. Unfortunately, in the mad world of family courts the solution is often nothing other than to yank the child from the mother as quickly as possible and thrust the child upon the father who has been accused of sexual abuse.

As demonstrated here, notwithstanding the evidence of sexual misconduct, this mental health expert enthusiastically supported steadily increasing overnight visits between the girl and her father that would likely culminate in a joint custodial arrangement. She categorically ruled out sex abuse, claiming that the mother's "enmeshment" with the child caused her to misperceive the father's actions. When she was asked at a hearing about evidence that the father had fully undressed in front of the young girl, the expert, according to the mother, purportedly did not seem to find this terribly relevant. In this way, "enmeshment" served the same purpose in this case as PAS served in so many others: it gave a key expert an excuse for ignoring evidence of alleged sexual abuse and blaming whatever could not be ignored on some putative fault of the child's mother.

"Enmeshment" Theories Gain Ground in Child Custody Cases

Although one may be tempted to dismiss the "enmeshment" theories peddled in child custody litigation as patently foolish, their effect in the world of family court can be serious indeed. To begin with, those who vociferously advocate for Richard Gardner's discredited PAS have broadly incorporated the term "enmeshment" when discussing "parental alienation" as these two concepts are presumed to go hand in hand. Daniel J. Rybicki, in a paper titled "Parental Alienation and Enmeshment Issues in Child Custody Cases," stated that in "intense conflict over custody issues, the potential is fairly large for the undercurrents of tension and conflict to produce conditions in which clinicians will report alienation and enmeshment concerns within the family unit. The intense bitterness which follows from such high conflict divorce may fuel the resulting disturbed communications which in turn yields various forms of interference with visitation and undermining of respect for the noncustodial parent" (emphasis supplied).¹²⁴

More recently, researchers have attributed such importance to “enmeshment” theories in child custody litigation that a much finer granular breakdown has been accorded to this phenomenon. In 2021, psychologist Benjamin D. Garber, in an article appearing in the *Journal of the American Academy of Matrimonial Lawyers*, analyzed in painstaking detail the “far more nuanced descriptions of three forms of enmeshment (i.e., adultification, parentification, and infantilization) . . . [in] consideration of how these destructive dynamics can be recognized and remedied.”¹²⁵

Protective Mother Prejudiced by “Enmeshment” Characterizations of Her Behavior

The ascription of labels, such as “enmeshment,” to this protective mother who tried to shield her child from what she suspected was sex abuse, had an effect on her case. Although the outcome (namely generous visitation for the father) was not as severe as other cases, it is perhaps still too early in the litigation process to predict how it will end. That is, at this relatively early juncture (a year or two into family court litigation) it’s very hard to forecast the outcome.¹²⁶

However, one thing is certain: the mother was prejudiced by the “enmeshment” diagnosis whereas the father was given a clean bill of health. The experts’ own words—“children need to grow away from us”—served as an eerie portent of what might come (that is, the removal of the child from the mother) if a judge were to accept the expert’s opinion at face value.

Words have an impact, especially when uttered in court. In this case the expert, in freely using “enmeshment” to describe the mother’s relationship with her young daughter, defined this pernicious phenomenon as an “intense need for closeness” with a child. One can theoretically imagine how, in the mind of a court, removing the child from the mother

would serve as a quick remedy for the “enmeshment”—not unlike how children have been (and continue to be) taken away from mothers who presumably shows signs of PAS.

“Resist and Refuse Dynamics”—and How It May Be More Insidious than Parental Alienation Syndrome

The next case narrative describes the case of a Brooklyn mother who lost sole custody of her six-year-old son because a mental health expert, chosen by the father, invoked a permutation of PAS, declaring there was a “danger” to the child because of the mother’s display of “resist and refuse dynamics with [the child] toward his father.”¹²⁷ When asked in court to clarify this concept, the expert stated: “It’s many people who work in the divorce and forensic area [that] are preferring this term [resist and refuse dynamics] to, and basically has similar meaning to, parental alienation but it’s being preferred because it’s more descriptive . . . and doesn’t imply a presumption of what the cause of the problem is.” The expert immediately went on to assert that the child was showing “what I call embryonic or early signs of attitudes and statements that are more intensified resist and refuse dynamics to come.”¹²⁸

So, although similar to PAS pseudoscience, this permutation, “Resist and Refuse Dynamics” (R-RD), has the added dimension of an extraordinary prognostic capability of detecting from the very early signs of R-RD how such dynamics will pick up steam over time. Armed with a crystal ball, the expert can supposedly tell, from the very early signs of parental alienation, what will happen in the relatively distant future.

Another advantage of this theory over the old PAS, according to the expert, is that although R-RD is “essentially the same phenomenon [as PAS]” it is also “an attempt to be more descriptive and *less presumptive of what the cause of the behavior is*” (emphasis supplied).¹²⁹ It is axiomatic in any profession that one routinely looks for “causes” as the basis of

a supposition of what is going on. Prosecutors, for example, routinely look for motive/causes to understand why a crime has been committed. Medical practitioners, likewise, look for causes, often referred to as etiology, to understand why the patient presents with a certain set of symptoms. By searching for causes, and entertaining sometimes countervailing causes, a diagnostician performs a robust analytic assessment of the situation. In contrast, if suddenly “causes” of conditions were to be completely eliminated from diagnostic work, one could opine just about anything under the sun without needing to anchor the hypothesis to a reasonable cause.

Thus, employing a theory like R-RD—designed to operate without reference to underlying causes—to describe the dynamic of a woman’s relationship to her partner or ex-partner establishes a dangerous precedent, because it allows a diagnostic label to be attached to a mother without any analysis of what might be driving the behavior that triggers the use of the label.

What this means is that one could assign blame to the mother by invoking the loosest of diagnostic requirements, that do not involve inquiry into a possible history of sexual abuse between the father and his (and the mother’s) child. That is, simply by virtue of one’s gender a mother could be accused of “resist and refuse dynamics” because this new permutation of PAS is intended to be “less presumptive” (less convincing or certain) of its origin/cause and, at the same time, “more descriptive” of the supposedly malicious posture of the mother toward the child’s father. This is weak science at best; and perhaps contrived and dangerous at worst. In fact, from a semantic point of view, the use of the term “resist”—as in “resisting arrest,” “resist, obstruct, delay of a peace officer,” and so forth—connotes in a hostile family court environment, *criminal* conduct. So, it is not hard to fathom how R-RD as a variant of PAS is even more punishing than its prototype.

Father's Social Service Record—Having an “Indicated” Finding—Wholly Disregarded by the Family Court because of “Resist and Refuse Dynamics” Used against the Mother

The history of this Brooklyn case is telling. Just two years before the mother lost custody as a result of the mental health expert's finding that the mother demonstrated R-RD—serving as a modern-day variant of the traditional diagnosis of PAS—the New York City Administration for Children's Services (which serves as the local CPS agency) had issued an official “Court-Ordered Investigation” report with an “indicated” finding against the father for “inadequate guardianship.” No finding was made against the mother as she was deemed a fit parent by this local CPS agency conducting the court-ordered investigation.

The report was comprehensive and clear. It stated that although the agency couldn't make a finding against the father for sexual abuse—the child, then only three years of age, was unable to confirm the abuse and there was no other corroborative evidence—the agency did find the father liable for inadequate guardianship (a form of neglect) “due to child expressing fear of his father, bed wets [*sic*], upset/agitated upon father's name mentioned . . . and that child reported his father hurts him, that he does not want to see his father, and that the child places his hands on his front private [parts] to show where his father hurts him.”¹³⁰

Mother Severely Prejudiced by “Resist and Refuse Dynamics”—and Stripped of Custody

Emboldened by the mental health expert's diatribe against the mother for alleged alienation of the child from the father—even though under cross examination the expert admitted that the mother was totally compliant with the terms of the parents' visitation agreement for making the child available to the father and couldn't identify instances that would

have been “contributing to the resist and refuse dynamics”³¹—the judge condemned the mother when she rendered her decision transferring sole custody to the father.

In the thirty-three-page written decision to take the young boy away from the mother and give sole custody of the child to the father, with limited visitation privileges for the mother, the judge repeatedly questioned the mother’s “decision-making process and ability to put [the child’s] best interests ahead of her own.”¹³² Throughout the decision, in fact, the judge accused the mother of making a knowingly false charge of sex abuse, even though the finding of a court-ordered investigation—described earlier—that the father was guilty of “inadequate guardianship” established at least the mother’s good faith in suspecting abuse. (The failure of a three-year-old to clearly confirm the abuse hardly proved the mother guilty of a knowingly false accusation.) Then, at the end of the decision, came kudos for the mental health expert, who had touted the modern-day variant of the much discredited PAS. Quoting his written report condemning the mother, the judge justified her ruling, which transferred custody of the young child to the father the boy (according to the court-ordered investigation) had accused of hurting him, with these words:

This conclusion is supported by [the mental health expert’s] findings—which this Court credits and gives weight to—that [the mother] has “several enduring, problematic personality traits” that negatively affect her parenting of [the child] and that “there is some degree of risk” that she will make other allegations of abuse against the father “with potentially dire adverse impact upon [the child’s] relationship with his father.”¹³³

One would hardly guess from the judge’s decision that this mother’s mental health had never been questioned—not until she was “examined” by the mental health “expert” chosen by the father and well versed in family court quackery. The mother was a biochemistry major at a

good school and was on track to become a medical doctor. Her litigation costs, however, forced her to leave school and instead work twelve-hour shifts in an urgent care facility. Sadly, the biased expert's testimony became fodder for the father. Because the expert, in addition to testifying at the trial, had already given testimony that was prejudicial to the mother at a pre-trial hearing, this gave the father the opportunity to tell the New York City Administration for Children's Services (which had been ordered by the court to perform an investigation of the parents and the child) that the child's mother had a "mental health diagnosis of [a] personality disorder,"¹³⁴ repeating what he learned from this mental health expert peddling the variants of PAS.

As it turned out, the investigators for the New York City Administration for Children's Services (ACS) found nothing of the kind. They spoke to the mother's therapist, who confirmed that the mother had no mental illness history and had not even been prescribed psychotropic medication. Instead, she sought episodic counseling due to a prior "life traumatic situation" and "is deemed stable"—and not in need of frequent therapy sessions because of her "demonstration of progress."¹³⁵ Yet, the bogus mental illness labels that had been ascribed to the mother during her family court litigation (i.e., a purported "personality disorder") allowed her ex-partner to discredit her. On the other hand, the ACS investigators did uncover troubling elements of the *father's* history (described below), casting additional doubt on the judge's decision to award custody to the father.

Father's Troubling Psychiatric History (Type 1 Bipolar Disorder—Resulting in Hospitalization after Trying to Jump off the Brooklyn Bridge) of No Consequence after Mother Discredited by Quack Theories

The ACS report showed that the father had been diagnosed with Type I Bipolar Disorder (the rarer and more serious of the major two “types”)—defined as including at least one major manic episode that often requires confinement to a mental hospital. The report by the city investigators showed that about eight years prior to the litigation (and before the parents had ever met) the father suffered a major “mood episode”³⁶—the details of which were filled in by the mother.

She explained to ACS that her child's father had told her that, having forgotten to take his psychotropic medication for his bipolar disorder, he had nearly jumped off the Brooklyn Bridge. The police rescued him and brought him to a New York psychiatric hospital where he stayed for over a month. Notwithstanding this serious episode, the report quoted the clinician supposedly monitoring the father's condition as dismissing the likelihood of a recurrence, noting that the father “is fully compliant with therapy and medication management.”¹³⁷ The mother, understandably, was not so confident. But her concerns were turned against her by the family court judge, under the influence of a tendentious mental health theory—leaving the mother with nowhere to turn.

Alas, this case is not an anomaly. A psychiatric history, cocaine usage, alcohol abuse, illegal possession of firearms and/or unsecured guns in the home, possession of semi-automatic assault weapons, DUIs, disorderly conduct, resisting an arrest, and so forth do not necessarily weaken a father's chance of winning custody of the children. By contrast, even a mother with no history of mental illness or substance abuse, and with a spotless legal record, can easily lose custody thanks to the effects of junk science on a family court proceeding.